Health sample: coverage, cost barriers to care and disability, by group, region and state

model-generated prediction, not a measurement

Model forecasts, by state (where the source carries state) and national group, of 3 questions from the National Health Interview Survey (NHIS) and the Behavioral Risk Factor Surveillance System (BRFSS), for NHIS 2026 and BRFSS 2026, among all US adults 18 and over. Prepared by Nate Grotberg; forecasts made 2026-11-09 (UTC), report prepared 2026-11-09.

Aggregate estimates for research, planning and public use with their label and citation line; never described as a poll or a survey result. Not a measurement of today, and not an estimate of any policy's effect: the model sees only each source's own history.

- Forecast of: NHIS 2026 and BRFSS 2026, all US adults 18 and over
- Built from: public NHIS data from 2019, 2020, 2021, 2022, 2023, 2024 and 2025 (history through NHIS 2025); public BRFSS data from 2021, 2022, 2023, 2024 and 2025 (history through BRFSS 2025); and a population frame from the Census Bureau's American Community Survey 2023. No calibration map is applied to any figure here: every forecast and interval is the model's own output
- Method: a statistical model: multilevel regression with poststratification blended with gradient boosting, anchored to carry-forward more strongly the more cases a cell has (`ensemble_mean@0.4.0`)
- Sealed: 220 recorded rows, SHA-256 `d56b1afbac275b4e…` (NHIS); 360 recorded rows, SHA-256 `01932b77b6ff1609…` (BRFSS) (The sealed record, below)

Summary

Forecast for NHIS 2026 and BRFSS 2026, all US adults: share giving each answer. Model-generated prediction, not a measurement.

Question	Answer	Forecast, all adults	80% interval (%)	95% interval (%)	Last measured	Lowest state forecast	Highest state forecast
Health coverage: uninsured adults	yes	9%	8–10	7–11	9% (NHIS 2025)	n/a (no state)	n/a (no state)
Access to care: could not see a doctor because of cost	yes	13%	11–15	9–17	13% (BRFSS 2025)	Hawaii 7%	Texas 18%
Disability: a lot of difficulty with a basic activity	yes	10%	9–11	9–11	10% (NHIS 2025)	n/a (no state)	n/a (no state)

Source: Nate Grotberg, model-generated prediction of NHIS 2026 and BRFSS 2026 among all US adults 18 and over, from public NHIS 2019–2025 and BRFSS 2021–2025 data, made 2026-11-09; 80% interval shown. Not a poll or survey estimate, and not a measurement.

The backtest record behind these forecasts

Before a source was served, its model was run on past waves it had not seen: every item predicted from earlier waves only, then scored against what the wave measured, for all adults and every standard cut. The baseline is carry-forward (repeat the last wave, smoothed toward 50% in thin cuts), the cheap alternative any analyst has. Two numbers per run: the Brier skill of the served model against carry-forward (0 is level, positive is better, +0.10 removes a tenth of carry-forward's error), and how often its 80% intervals held the measured share, with carry-forward's in parentheses. Every figure below is restated from docs/predictors.md.

NHIS: the served model on held-out waves, each predicted from earlier waves only and scored cell by cell. A backtest, not a forecast.

Held-out wave	Brier skill against carry-forward	Lowest average top-box miss	80% intervals that held: model (carry-forward)	Defaults chosen on this wave
NHIS 2024 and 2025, pooled	+0.038 (95% interval +0.020 to +0.059)	the served model	83% (51%)	yes
NHIS 2023	+0.034	the served model	90% (56%)	no

Source: docs/predictors.md, "Serving NHIS (2026-10-28)". Skill: 1 – model Brier / carry-forward Brier; 0 is level, positive is better.

NHIS on points. The served model had the lowest average top-box miss over the two waves the default was chosen on, ahead of carry-forward by 0.012 points (95% interval 0.005 to 0.018), and the lowest again on NHIS 2023, which no default was chosen on. A hundredth of a point is a small edge: on points the model is close to repeating the last wave, and the larger difference is in its intervals.

Selection exposure. The served model's settings were chosen by reading the NHIS 2024 and 2025 runs, so those are selection-exposed: their skill is an optimistic estimate of what a forward forecast should expect, not out-of-time evidence (the registry is `prior_predictors.selection_exposure`). The NHIS 2023 run came after that choice and is not exposed. The NHIS 2024 wave is also availability-exposed: the population frame its run used was published after that wave's fieldwork began.

BRFSS: the served model on held-out waves, each predicted from earlier waves only and scored cell by cell. A backtest, not a forecast.

Held-out wave	Brier skill against carry-forward	Lowest average top-box miss	80% intervals that held: model (carry-forward)	Defaults chosen on this wave
BRFSS 2024	+0.001	the served model	95% (41%)	yes
BRFSS 2023	–0.005	the served model	93% (30%)	yes

Source: docs/predictors.md, "Serving five more sources (2026-10-29)". Skill: 1 – model Brier / carry-forward Brier; 0 is level, positive is better.

BRFSS on points. Level with repeating the last wave on points (a source that barely moves year to year is hard to beat); the case for the model is its intervals.

Selection exposure. The served model's settings were chosen by reading the BRFSS 2023 and 2024 runs, so those are selection-exposed: their skill is an optimistic estimate of what a forward forecast should expect, not out-of-time evidence (the registry is `prior_predictors.selection_exposure`). No BRFSS run in this record came after that choice. The BRFSS 2024 wave is also availability-exposed: the population frame its run used was published after that wave's fieldwork began.

Your custom report

This sample covers 3 catalog items. A custom report runs the same pipeline on the catalog items closest to your own study, forecast before the source publishes and sealed when made.

- **Scope.** Up to three catalog items on your issue, each for all adults, every state the source carries (all 50 and DC; the four Census regions where the public file has no state) and the national groups by age, sex, race and ethnicity, education and region, with 80% and 95% intervals beside the last measured share.
- **Deliverables.** A PDF report like this one and CSVs with every item, cut and answer, both intervals, the measured history and the basis of every figure.
- **Price and turnaround.** $5,000 per issue at the pilot price, 50% on signature; 10 business days from the cleared deposit; one revision of the delivered report included.
- **Not offered.** Groups within a state (such as women in Ohio): the engine makes one-variable cuts. Individual-level scores or lists of people, for any buyer.

What you may do with these numbers

Aggregate estimates by state and national group, for reports, strategy and message testing, and for public use (testimony, press releases, op-eds, member emails) with the label “model-generated prediction, not a measurement” and the citation line below on every use. Never individual-level scores, lists of people or voter-file matching; never described as a poll; never used in paid political ads or candidate campaign material.

How to cite

Source: Nate Grotberg, model-generated prediction of NHIS 2026 and BRFSS 2026 among all US adults 18 and over, from public NHIS 2019–2025 and BRFSS 2021–2025 data, made 2026-11-09; 80% interval shown. Not a poll or survey estimate, and not a measurement.

How to read the tables

- **Forecast**: the model's forecast, made 2026-11-09, of the share of US adults in that state or group who give the answer named, as the source will measure it in its next wave (NHIS 2026 and BRFSS 2026). It is not a measurement of today.
- **80% and 95% intervals**: nominal model ranges targeting coverage four times out of five, and 19 times out of 20. The cited federal coverage checks are on national and demographic cuts. State intervals are nominal.

In a held-out test of 18 questions across every state the wave carries, BRFSS 2025 state 80% intervals held 90% of state shares (95%: 96%).

- **Measured**: the weighted share the source recorded in its last wave (NHIS 2025 and BRFSS 2025), with its number of cases and, in parentheses, its effective cases (the sample size its weights are worth). It is the starting point for carry-forward, the baseline beside each forecast in the CSVs (`carry_forward_pct`).
- **Sample**: *thin* means fewer than 100 effective cases in the state in the source's last wave, *small* 100 to 199. Read a thin state's forecast as a range, not a point.
- Percentages are whole numbers rounded half up once from unrounded values; states.csv and groups.csv carry two decimals, rounded the same way, and the whole-percent bounds the tables use (`*_printed`). states.csv also reads each interval against 50% on those printed bounds.

Health coverage: uninsured adults. Model forecast for NHIS 2026, share answering “yes”

Your question, as written: “Uninsured at the time of interview: no private health insurance, Medicare, Medicaid, CHIP, state-sponsored plan, other government program or military plan (NCHS's coverage-status recode NOTCOV_A, the definition used in Health, United States).”

Catalog item: `uninsured_now`, “Uninsured at the time of interview: no private health insurance, Medicare, Medicaid, CHIP, state-sponsored plan, other government program or military plan (NCHS's coverage-status recode NOTCOV_A, the definition used in Health, United States).” (NHIS 2025 item `NOTCOV_A`; asked in NHIS 2019, 2020, 2021, 2022, 2023, 2024 and 2025). Wording match with your question: **exact**.

Answer reported: `yes`, of `yes`, `no`. Shares exclude skipped answers.

Basis of every figure in this section: measured history. Extends what earlier waves of this same question measured in this same group.

Forecast for NHIS 2026, all adults (model-generated prediction, not a measurement): **9%** answering yes (80% interval 8–10; 95% interval 7–11). NHIS 2025 measured 9% on 23,992 cases.

No state. The public NHIS file carries Census region only, so this item is forecast for all adults and the national groups, the four regions included, and for no state.

National groups

Health coverage: uninsured adults. Model forecast for NHIS 2026, all adults and national groups, share answering yes. Model-generated prediction, not a measurement; all US adults 18 and over.

Group	Forecast	80% interval (%)	95% interval (%)	NHIS 2025 measured	NHIS 2025 cases (effective)
All adults	9%	8–10	7–11	9%	23,992 (16,422)
Age					
18–29	14%	12–17	11–18	14%	2,844 (2,098)
30–39	13%	12–15	11–17	14%	3,737 (2,836)

Group	Forecast	80% interval (%)	95% interval (%)	NHIS 2025 measured	NHIS 2025 cases (effective)
40–49	12%	10–13	9–15	11%	3,463 (2,628)
50–59	8%	6–9	6–10	7%	3,355 (2,555)
60–69	4%	4–5	3–6	4%	4,569 (3,551)
70 and over	1%	1–1	0–1	1%	6,024 (4,459)
Sex					
Men	10%	9–12	8–13	10%	11,129 (7,788)
Women	8%	7–9	6–10	8%	12,863 (8,647)
Race and ethnicity					
White, non-Hispanic	6%	5–7	5–8	6%	15,905 (11,019)
Black, non-Hispanic	9%	8–11	7–12	9%	2,689 (1,805)
Hispanic	20%	17–23	16–25	20%	3,341 (2,401)
Other or multiracial, non-Hispanic	7%	5–8	5–9	6%	2,057 (1,471)
Education					
No high school diploma	23%	20–26	19–27	24%	1,790 (1,221)
High school graduate	12%	10–14	9–15	12%	6,053 (4,057)
Some college	8%	7–9	6–10	8%	6,625 (4,472)
Four-year degree	4%	3–5	3–5	4%	5,702 (4,264)
Postgraduate degree	2%	2–3	2–3	2%	3,822 (2,991)
Region					
Northeast	5%	5–6	4–7	5%	4,353 (2,943)
Midwest	8%	6–9	6–9	8%	5,210 (3,576)
South	12%	10–14	10–15	12%	8,731 (6,170)
West	8%	6–9	6–10	8%	5,698 (3,756)

Forecasts: model forecast for NHIS 2026. Measured: NHIS 2025, weighted. Model-generated prediction, not a measurement. Source: Nate Grotberg, model-generated prediction of NHIS 2026 and BRFSS 2026 among all US adults 18 and over, from public NHIS 2019–2025 and BRFSS 2021–2025 data, made 2026-11-09; 80% interval shown. Not a poll or survey estimate, and not a measurement.

Access to care: could not see a doctor because of cost. Model forecast for BRFSS 2026, share answering “yes”

Your question, as written: “Was there a time in the past 12 months when you needed to see a doctor but could not because you could not afford it?”

Catalog item: `cost_barrier_to_care`, “Was there a time in the past 12 months when you needed to see a doctor but could not because you could not afford it?” (BRFSS 2025 item `MEDCOST1`; asked in BRFSS 2021, 2022,

2023, 2024 and 2025). Wording match with your question: **exact**.

Answer reported: `yes`, of `yes`, `no`. Shares exclude skipped answers.

Basis of every figure in this section: measured history. Extends what earlier waves of this same question measured in this same group.

Forecast for BRFSS 2026, all adults (model-generated prediction, not a measurement): **13%** answering yes (80% interval 11–15; 95% interval 9–17). BRFSS 2025 measured 13% on 347,098 cases.

State forecasts

Access to care: could not see a doctor because of cost. Model forecast for BRFSS 2026, all 50 states and DC, share answering yes. Model-generated prediction, not a measurement.

State	Forecast	80% interval (%)	95% interval (%)	BRFSS 2025 measured	BRFSS 2025 cases (effective)	Sample
Texas	18%	14–21	13–24	18%	6,160 (2,860)	
Georgia	16%	13–19	11–21	16%	6,140 (3,101)	
Mississippi	16%	13–19	11–22	n/a	n/a	
Oklahoma	15%	12–18	11–20	15%	4,147 (2,883)	
Arkansas	15%	12–18	11–20	15%	5,067 (2,829)	
Nevada	15%	11–18	10–21	n/a	n/a	
Alabama	14%	12–18	10–20	15%	4,146 (2,475)	
Louisiana	14%	12–17	11–19	15%	4,859 (3,111)	
Florida	14%	11–17	10–18	14%	13,288 (3,430)	
Colorado	14%	11–16	10–18	14%	9,362 (6,825)	
Tennessee	13%	11–16	10–18	13%	5,679 (3,134)	
Arizona	13%	11–16	10–18	14%	5,326 (2,049)	
New Jersey	13%	11–16	10–17	13%	6,638 (3,110)	
Wyoming	13%	10–16	9–18	13%	4,521 (2,568)	
Illinois	13%	11–16	10–17	13%	11,176 (6,880)	
Missouri	12%	10–15	9–17	13%	5,892 (3,194)	
Utah	12%	10–15	9–17	12%	11,158 (6,311)	
Montana	12%	10–15	9–17	13%	5,812 (3,763)	
West Virginia	12%	10–15	9–17	12%	4,197 (2,798)	
South Carolina	12%	9–15	8–17	12%	4,095 (1,139)	
Indiana	12%	10–15	9–17	12%	6,479 (3,152)	
New Mexico	12%	10–15	9–16	13%	3,390 (1,720)	
Alaska	12%	10–15	8–17	12%	5,468 (2,489)	
Kentucky	12%	10–15	9–16	12%	7,395 (3,368)	

State	Forecast	80% interval (%)	95% interval (%)	BRFSS 2025 measured	BRFSS 2025 cases (effective)	Sample
Oregon	12%	9–14	9–16	12%	4,664 (3,409)	
Ohio	12%	10–14	9–16	12%	8,914 (4,012)	
Idaho	12%	9–15	8–16	12%	3,548 (2,150)	
Washington	12%	10–14	9–16	12%	19,260 (11,654)	
Kansas	11%	9–14	8–16	11%	9,854 (5,470)	
Connecticut	11%	9–14	9–15	12%	7,365 (4,191)	
Delaware	11%	9–14	8–15	12%	4,167 (2,015)	
North Carolina	11%	9–14	8–15	11%	3,477 (2,352)	
Nebraska	11%	9–14	8–15	11%	6,933 (3,414)	
New York	11%	9–14	8–15	11%	14,625 (8,242)	
Maryland	11%	9–13	8–15	11%	12,819 (5,938)	
California	11%	9–13	8–15	n/a	n/a	
Pennsylvania	11%	9–13	8–15	11%	2,991 (1,593)	
Michigan	11%	9–13	8–15	11%	10,061 (5,492)	
Rhode Island	11%	9–13	8–14	11%	5,366 (2,958)	
Wisconsin	11%	9–13	8–14	11%	11,607 (5,623)	
Minnesota	10%	8–13	8–14	10%	14,504 (8,506)	
South Dakota	10%	8–13	7–14	10%	4,095 (1,331)	
Virginia	10%	8–12	7–14	10%	4,977 (2,092)	
Maine	10%	8–12	7–13	10%	12,105 (6,056)	
North Dakota	9%	7–12	7–13	9%	5,685 (3,247)	
District of Columbia	9%	7–12	6–13	9%	2,596 (1,415)	
New Hampshire	9%	7–11	7–13	9%	5,949 (2,219)	
Massachusetts	9%	7–11	6–12	9%	9,383 (5,629)	
Iowa	9%	7–11	6–12	9%	8,018 (5,999)	
Vermont	8%	7–10	6–11	8%	6,421 (3,656)	
Hawaii	7%	6–9	5–10	7%	7,319 (3,989)	

Forecasts: model forecast for BRFSS 2026. Measured: BRFSS 2025, weighted. Model-generated prediction, not a measurement. Source: Nate Grotberg, model-generated prediction of NHIS 2026 and BRFSS 2026 among all US adults 18 and over, from public NHIS 2019–2025 and BRFSS 2021–2025 data, made 2026-11-09; 80% interval shown. Not a poll or survey estimate, and not a measurement.

Thin states. No state shown has fewer than 100 effective BRFSS 2025 cases.

No BRFSS 2025 cases. BRFSS 2025 holds no cases for Mississippi, Nevada and California, so their forecasts stand on earlier waves and on how people of the same age, sex, race and ethnicity, and education answer

elsewhere.

National groups

Access to care: could not see a doctor because of cost. Model forecast for BRFSS 2026, all adults and national groups, share answering yes. Model-generated prediction, not a measurement; all US adults 18 and over.

Group	Forecast	80% interval (%)	95% interval (%)	BRFSS 2025 measured	BRFSS 2025 cases (effective)
All adults	13%	11–15	9–17	13%	347,098 (80,234)
Age					
18–29	17%	14–21	13–23	17%	40,066 (12,557)
30–39	17%	15–21	13–23	18%	39,715 (11,739)
40–49	16%	13–19	12–21	16%	44,681 (11,386)
50–59	13%	11–16	10–18	13%	51,016 (12,383)
60–69	9%	7–10	6–12	9%	68,394 (15,600)
70 and over	3%	2–4	2–5	3%	103,226 (20,935)
Sex					
Men	11%	9–14	8–15	11%	164,169 (40,390)
Women	14%	12–17	11–18	14%	182,929 (39,908)
Race and ethnicity					
White, non-Hispanic	10%	8–12	7–13	10%	262,766 (69,402)
Black, non-Hispanic	15%	12–18	11–21	15%	26,009 (7,885)
Hispanic	20%	17–24	15–27	21%	30,914 (7,089)
Other or multiracial, non-Hispanic	14%	12–17	11–19	15%	27,409 (6,861)
Education					
No high school diploma	22%	18–26	16–28	22%	19,412 (4,775)
High school graduate	14%	12–17	11–19	14%	90,010 (21,573)
Some college	13%	11–16	10–18	13%	92,131 (22,628)
Four-year degree or more	8%	6–10	6–11	8%	145,545 (36,779)
Region					
Northeast	11%	9–14	8–15	11%	70,843 (16,933)
Midwest	12%	10–14	9–16	12%	103,218 (40,137)
South	14%	12–17	10–19	14%	93,209 (23,697)
West	12%	10–15	9–17	12%	79,828 (26,546)

Forecasts: model forecast for BRFSS 2026. Measured: BRFSS 2025, weighted. Model-generated prediction, not a measurement. Source: Nate Grotberg, model-generated prediction of NHIS 2026 and BRFSS 2026 among all US adults

18 and over, from public NHIS 2019–2025 and BRFSS 2021–2025 data, made 2026-11-09; 80% interval shown. Not a poll or survey estimate, and not a measurement.

Disability: a lot of difficulty with a basic activity. Model forecast for NHIS 2026, share answering “yes”

Your question, as written: “Disability by the Washington Group Short Set: a lot of difficulty, or cannot do at all, in at least one of seeing, hearing, walking or climbing steps, communicating, remembering or concentrating, and self-care (NCHS recode DISAB3_A).”

Catalog item: `disability_status_wg`, “Disability by the Washington Group Short Set: a lot of difficulty, or cannot do at all, in at least one of seeing, hearing, walking or climbing steps, communicating, remembering or concentrating, and self-care (NCHS recode DISAB3_A).” (NHIS 2025 item `DISAB3_A`; asked in NHIS 2019, 2020, 2021, 2022, 2023, 2024 and 2025). Wording match with your question: **exact**.

Answer reported: `yes`, of `yes`, `no`. Shares exclude skipped answers.

Basis of every figure in this section: measured history. Extends what earlier waves of this same question measured in this same group.

Forecast for NHIS 2026, all adults (model-generated prediction, not a measurement): **10%** answering yes (80% interval 9–11; 95% interval 9–11). NHIS 2025 measured 10% on 24,058 cases.

No state. The public NHIS file carries Census region only, so this item is forecast for all adults and the national groups, the four regions included, and for no state.

National groups

Disability: a lot of difficulty with a basic activity. Model forecast for NHIS 2026, all adults and national groups, share answering yes. Model-generated prediction, not a measurement; all US adults 18 and over.

Group	Forecast	80% interval (%)	95% interval (%)	NHIS 2025 measured	NHIS 2025 cases (effective)
All adults	10%	9–11	9–11	10%	24,058 (16,462)
Age					
18–29	6%	5–6	4–7	6%	2,862 (2,114)
30–39	5%	4–6	4–6	5%	3,753 (2,844)
40–49	7%	6–8	6–8	7%	3,471 (2,634)
50–59	10%	9–11	8–11	10%	3,364 (2,564)
60–69	12%	11–13	11–14	12%	4,577 (3,557)
70 and over	21%	20–23	19–24	21%	6,031 (4,464)
Sex					

Group	Forecast	80% interval (%)	95% interval (%)	NHIS 2025 measured	NHIS 2025 cases (effective)
Men	9%	8–10	8–10	9%	11,161 (7,809)
Women	10%	10–11	9–12	10%	12,897 (8,666)
Race and ethnicity					
White, non-Hispanic	11%	10–12	10–12	11%	15,934 (11,032)
Black, non-Hispanic	11%	9–12	9–12	11%	2,700 (1,813)
Hispanic	8%	7–9	6–9	7%	3,364 (2,420)
Other or multiracial, non-Hispanic	7%	6–9	6–9	7%	2,060 (1,471)
Education					
No high school diploma	19%	17–21	16–22	19%	1,801 (1,229)
High school graduate	12%	11–14	11–14	12%	6,077 (4,070)
Some college	10%	9–11	9–11	10%	6,641 (4,484)
Four-year degree	5%	5–6	4–6	5%	5,714 (4,273)
Postgraduate degree	5%	4–5	4–6	5%	3,825 (2,992)
Region					
Northeast	9%	8–10	8–11	9%	4,361 (2,948)
Midwest	11%	10–12	9–12	11%	5,219 (3,581)
South	11%	10–12	10–12	11%	8,766 (6,192)
West	8%	7–9	7–9	8%	5,712 (3,764)

Forecasts: model forecast for NHIS 2026. Measured: NHIS 2025, weighted. Model-generated prediction, not a measurement. Source: Nate Grotberg, model-generated prediction of NHIS 2026 and BRFSS 2026 among all US adults 18 and over, from public NHIS 2019–2025 and BRFSS 2021–2025 data, made 2026-11-09; 80% interval shown. Not a poll or survey estimate, and not a measurement.

Limits

- All US adults 18 and over, and one-variable cuts only: no groups within a state (such as women in Ohio).
- A forecast of what each source will measure in its next wave (NHIS 2026 and BRFSS 2026): not a measurement of today, and not a survey.
- The model has no state-level predictor (such as a state's policies or its economy). Each state gets one partially pooled effect on top of age, sex, race and ethnicity, and education, so in a thin state a distinctive result is pulled toward what its demographics predict nationally.
- One time trend serves every state and group: the model cannot forecast a shift in one place that the others do not share.
- Events and policy changes since each source's last wave reach the forecast only through that trend; the model makes no adjustment for any of them.
- NHIS: the public file carries Census region only, so those items have no state figures.

The sealed record

Every row behind this report from NHIS, 220 in all from `carry_forward@0.1.0`, `demographics_only@0.1.0`, `ensemble_mean@0.4.0`, `gbm_pooled@0.1.0`, `mrp_numpyro@0.5.0`, was recorded under a private tenant on a scratch copy of the ledger and sealed at 2026-11-09 22:57 UTC (`seal-nhis.json`, SHA-256 `d56b1afbac275b4e83b8e04691b45fd3006e9b5abda2cf32e8fdc30ba92812b5`). The raw distribution of each sealed row, the predictor's own output and what this report prints, is sealed in a supplement (`raw-seal-nhis.json`, SHA-256 `2adba997423f42e784dcaf8496777a11f675a52f97b360648897ed1272e81da2`). The seal names code commit `5d991f7edcb3`. An OpenTimestamps proof sits beside each stamped file; it dates the file by a Bitcoin block once a calendar's attestation is upgraded. Readers cannot check either until the seal files and rows are released.

Every row behind this report from BRFSS, 360 in all from `carry_forward@0.1.0`, `demographics_only@0.1.0`, `ensemble_mean@0.4.0`, `gbm_pooled@0.1.0`, `mrp_numpyro@0.5.0`, was recorded under a private tenant on a scratch copy of the ledger and sealed at 2026-11-09 22:57 UTC (`seal-brfss.json`, SHA-256 `01932b77b6ff1609d1c0bef2829e535eff19adca5be3b947f3d7dd304fd838c3`). The raw distribution of each sealed row, the predictor's own output and what this report prints, is sealed in a supplement (`raw-seal-brfss.json`, SHA-256 `74605f268255573f811ac55b8732648ff2a93a1b94d185e28b974efa83302a98`). The seal names code commit `5d991f7edcb3`. An OpenTimestamps proof sits beside each stamped file; it dates the file by a Bitcoin block once a calendar's attestation is upgraded. Readers cannot check either until the seal files and rows are released.

Sources

- National Center for Health Statistics. National Health Interview Survey, 2019-2025. Public-use data file and documentation. https://www.cdc.gov/nchs/nhis/documentation/index.html. Data Source: National Center for Health Statistics, National Health Interview Survey. The analyses, interpretations and conclusions are the author's, not NCHS's. (NHIS documentation).
- Centers for Disease Control and Prevention (CDC). Behavioral Risk Factor Surveillance System Survey Data, 2021, 2022, 2023, 2024, 2025. Atlanta, Georgia: U.S. Department of Health and Human Services, Centers for Disease Control and Prevention. https://www.cdc.gov/brfss/ (BRFSS documentation).
- US Census Bureau, American Community Survey 2023 public-use microdata: the population frame the forecasts are weighted to.

The forecasts, analyses and conclusions in this report are Nate Grotberg's alone. The National Center for Health Statistics, the Centers for Disease Control and Prevention and the Census Bureau produced the source data and are responsible only for those data. Their use here implies no endorsement of this report or its author by any of them or by the U.S. Government. The public-use data are available at no charge from each agency's website.

Model-generated prediction, not a measurement. Source: Nate Grotberg, model-generated prediction of NHIS 2026 and BRFSS 2026 among all US adults 18 and over, from public NHIS 2019–2025 and BRFSS 2021–2025 data, made 2026-11-09; 80% interval shown. Not a poll or survey estimate, and not a measurement.